

BURNS

1. Case definition

Identifying the degree of burn :

First degree : pain +++, no vesicles.

Second degree: pain +++, phlyctenes, red surface on skin.

Third degree: less pain, skin removed, whitish surface.

Fourth degree : no pain in specific areas, black surface

carbonized).

Note:

1. The importance of identifying the degree of burn lies in the fact that contractures must be avoided, and that third-degree burns heal much faster with a simple skin graft.
2. It is often difficult to judge with certainty whether a burn is second- or third-degree.
3. Burns often present in mixed degrees.
4. After a few days, the situation often becomes clearer.

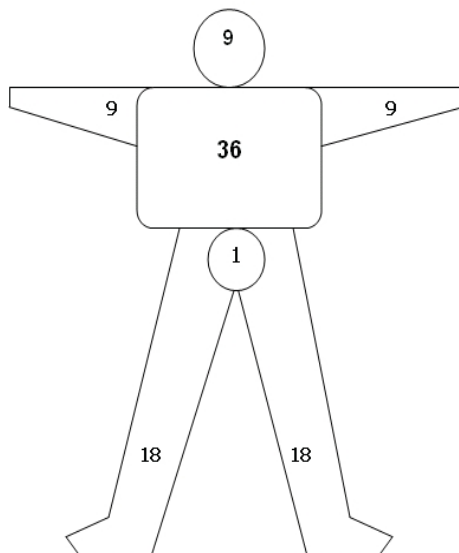
Second-degree surfaces regenerate the skin within the surface itself.

In the case of third-degree burns, the skin must necessarily regenerate from the edges of the wound. In most cases, this results in ugly scars.

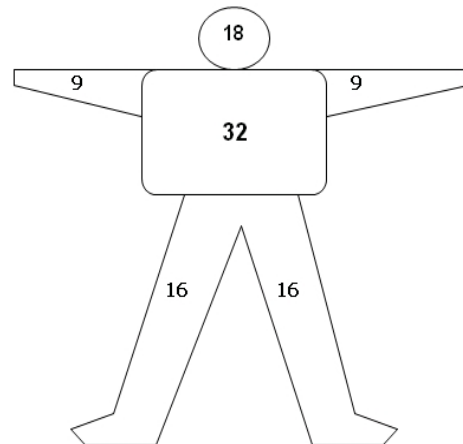
2. Support

Use Wallace's area calculation to estimate the affected area

CALCUL DE SURFACE
Règle de 9 WALLACE



ADULT



CHILD

Management of burns according to severity and complication

Very fresh burns (Whatever the surface affected)	Rinse immediately fresh clean water (until pain subsides)
Severe burns <u>Children</u> 0 to 3 years: > 7 <u>Children</u> 4 to 10: > 10% . <u>Adult</u> > 15 % <u>Child with</u> third-degree burns, even if limited, on hands, wrists, elbow, neck, face, knee, genitals, armpits. <u>Children</u> with limited but third-degree burns, with red granulation of tissues	Vitamin A 1 dose REFER urgently See FT hypovolemic shock
MODERATE or LIGHT burn <u>Children</u> with restricted third-degree burns <u>Children and adults</u> with second-degree burns	* aspirate phlyctenes, gently excise dead skin (don't try remove everything the first time) * rinse clean water, remove foreign objects (traditional medicine) * cleaning Polyvidone iodine 10% ¼ diluted with ¾ 0.9% sodium chloride never alcohol) * vitamin A+ VAT (check vaccination status first) * bandage: apply a generous layer of silver sulfadiazine ointment, then cover with a thick layer of sterile compresses and a layer of sterile gauze. wrap loosely in gauze. Dressings are repeated every 2 days, after administering Paracetamol 500 mg 1x 2cps 60 min before dressing. * weigh the child and record weight * Information-Education-Communication (IEC) on hygiene and the need increase calories diet (vegetable oil, several small meals) - see FT PCIME. * review after 3 days, or keep the child under observation for 24 hours and place under an impregnated mosquito net.
In case of superinfection (pus)	Cloxacillin 500mg 2x 2 capsules/day for 10 to 15 days